

Patient Scheduling — Vertical Slice Workflows

Workflows · Roles · Data Integrations · HITL Checkpoints · Baseline vs Agentic Metrics

Eight vertical slices. Each one is a buy-able outcome with its own evals, integrations, and human-in-the-loop boundaries. This deck shows what a customer is actually getting per slice — and where the agentic system replaces, augments, or escalates to a human.

8 SLICES

1 portfolio

Each independently buyable

5 LENSES

per slice

Workflow · roles · data · HITL · metrics

1 STAR

for the MVE

Slice 1 — the proof in 2 weeks

The five lenses on every slice

Every vertical slice is decomposed against the same five questions. Same lenses, same layout — so executives can compare slices apples-to-apples and see where the agentic system earns its keep.

1WORKFLOW	2USER ROLES	3DATA & SYSTEMS	4HITL POINTS	5METRICS
What happens, in order 5–6 numbered steps. HITL steps marked in amber.	Who is involved Patient, clinical, ops, and care-team roles per slice.	Integrations the agent reads/writes Epic FHIR, payer APIs, CRM, messaging, identity.	Where humans must stay in the loop Mandatory review, escalation, regulatory, edge cases.	Baseline vs with agentic Industry-typical baseline vs the bar we underwrite to.

READING THE METRICS BLOCK	
Baseline (without agentic)	With agentic system (the bar we underwrite to)
Industry-typical numbers from health-system benchmarks (HFMA, MGMA, AHRQ). Where ranges vary by setting (community vs academic), we show the band.	The eval-pass-rate target written into the contract. Meet the bar → milestone payment. Not aspirational marketing — the number we agree to be measured against.

★ Slot search + intent capture

The MVE star — the slice that proves the model in 2 weeks.

WORKFLOW



USER ROLES

-  **Patient**
Self-service via web/voice/SMS
-  **Front desk**
Oversees ambiguous cases
-  **Scheduler**
Handles complex/escalated
-  **Care coordinator**
Acuity escalations

HITL CHECKPOINTS

-  Ambiguous intent (>15% confidence gap) → scheduler review
-  Eligibility failure → benefits coordinator
-  High-acuity language detected → triage handoff

DATA & SYSTEM INTEGRATIONS

-  Epic FHIR — Appointment / Slot / Schedule
-  Eligibility 270/271 (clearinghouse)
-  CRM — patient profile + history
-  SMS · voice gateway · web chat
-  Identity / SSO · MFA

BASELINE vs WITH AGENTIC SYSTEM

BASELINE	WITH AGENTIC
Online self-service 35–45%	Online self-service 70–85%
Avg booking time 6–9 min (call)	Avg booking time <60 sec
Call abandonment 20–25%	Call abandonment <8%
After-hours capture <10%	After-hours capture 60%+

Reschedule + waitlist management

Recapture revenue from cancellations; serve the next-up patient first.

WORKFLOW



USER ROLES

-  **Patient**
Initiates change
-  **Waitlist patients**
Eligible to backfill
-  **Scheduler**
Waitlist policy + exceptions
-  **Practice manager**
Slot policy oversight

HITL CHECKPOINTS

-  Late cancel within policy window → manager review
-  Waitlist priority disputes → scheduler
-  Clinic-blocked / template slots → not auto-fill

DATA & SYSTEM INTEGRATIONS

-  Epic FHIR — Appointment status events
-  Waitlist registry (custom or vendor)
-  SMS · email · push
-  CRM — outreach history

BASELINE vs WITH AGENTIC SYSTEM

BASELINE	WITH AGENTIC
Waitlist fill rate 8–12%	Waitlist fill rate 35–55%
Cancelled-slot revenue loss 15–30%	Revenue recovery 60%+ of cancels
Same-day rebook Manual · slow	Same-day rebook 3× faster
Outreach-to-fill latency Hours–days	Latency to first offer <2 min

Symptom triage + routing

Highest-stakes slice — safety nets are mandatory, not optional.

WORKFLOW



USER ROLES

- Patient**
Symptom self-report
- RN / triage nurse**
Mandatory red-flag review
- PCP**
Same-day urgent slot
- Specialist**
Routed cases

HITL CHECKPOINTS

- Red-flag symptoms (chest pain / SOB / SI) → 100% nurse + 911 path
- Discordant self-report vs history → nurse review
- Pediatric / pregnancy / oncology → mandatory clinician sign-off

DATA & SYSTEM INTEGRATIONS

- Epic — clinical context · problem list
- Triage decision-support library
- CCDA / HL7 — clinical history
- SMS · voice · web

BASELINE vs WITH AGENTIC SYSTEM

BASELINE	WITH AGENTIC
<i>Triage call duration</i> 12–18 min	<i>Triage interaction</i> ~90 sec auto-route
<i>Nurse review rate</i> 100% of calls	<i>Nurse review</i> 15–20% (red-flag + audit)
<i>Mis-routing rate</i> 8–12%	<i>Mis-routing rate</i> <3%
<i>After-hours triage</i> Limited / outsourced	<i>After-hours coverage</i> 24x7 with SLA

Insurance + eligibility check

Stop downstream denials at the front door — pre-visit financial clarity.

WORKFLOW



USER ROLES

- Patient**
Provides insurance info
- Patient access rep**
Exception handling
- Benefits coordinator**
Denials + auths
- Billing**
Pre-service financial counsel

HITL CHECKPOINTS

- Eligibility denial / coverage gap → benefits coordinator
- Prior-auth required → care team queue
- Out-of-network → patient consent + financial counseling

DATA & SYSTEM INTEGRATIONS

- Payer APIs — 270/271 eligibility
- Epic registration · coverage records
- Prior-auth systems · payer portals
- Eligibility clearinghouse

BASELINE vs WITH AGENTIC SYSTEM

BASELINE	WITH AGENTIC
Eligibility check time 4–7 min	Eligibility check <30 sec
Denial / rework rate 18–25%	Denials / rework <8%
Cost per claim rework \$35–45	Cost per rework avoided Net positive
Pre-visit cost clarity Rare	Pre-visit clarity 90%+ of patients

Provider matching · continuity of care

Right provider, right language, right history — no orphan visits.

WORKFLOW



USER ROLES

- ☐ **Patient**
Preferences + history
- ☐ **PCP / specialist**
Panel rules + capacity
- ☐ **Scheduler**
Edge-case routing
- ☐ **Care coordinator**
New PCP assignment

HITL CHECKPOINTS

- ☐ New PCP assignment → care coordinator approval
- ☐ Panel-closed override → manager
- ☐ Specialty-edge cases (rare conditions) → scheduler

DATA & SYSTEM INTEGRATIONS

- Epic provider directory · panel mgmt
- CRM — language / preference profile
- Continuity-of-care records (CCDA)
- Specialty referral catalog

BASELINE vs WITH AGENTIC SYSTEM

BASELINE	WITH AGENTIC
PCP continuity rate 55–65%	PCP continuity 80–90%
Language / pref match 78%	Language / pref match 95%+
Time to find provider Manual lookup	Match-to-offer Seconds
Care-team awareness Inconsistent	Care-team continuity Tracked + measured

Reminders + no-show prevention

Targeted intervention — risk-score, then meet the patient where they are.

WORKFLOW



USER ROLES

-  **Patient**
Receives + responds
-  **Scheduler**
Standard reminder ops
-  **Social worker**
High-risk outreach
-  **Practice manager**
Cadence policy

HITL CHECKPOINTS

-  High-risk no-show flag → social worker outreach
-  Transport / childcare barriers → care navigator
-  Cultural / faith-based scheduling exceptions

DATA & SYSTEM INTEGRATIONS

-  SMS · voice · email · push
-  Epic Appointments · status sync
-  Social-determinants registry
-  Transport / childcare partner APIs

BASELINE vs WITH AGENTIC SYSTEM

BASELINE	WITH AGENTIC
No-show rate 15–30%	No-show rate 8–15% (30–50% redux)
Reminder design 1 touch · flat	Reminder design Multi-touch · channel-fit
Risk targeting None	Risk targeting Per-patient
Barrier resolution Reactive	Barrier resolution Proactive

Multi-language + accessibility

Equity is non-negotiable — clinical content needs human interpreters.

WORKFLOW



USER ROLES

- ☐ **Patient**
Preferred language + needs
- ☐ **Interpreter**
Mandatory for clinical content
- ☐ **Accessibility coord.**
ADA accommodations
- ☐ **Front desk**
Edge-case fallback

HITL CHECKPOINTS

- ☐ Clinical-content interpretation → human interpreter (regulatory)
- ☐ Ambiguous translation / dialect → human review
- ☐ ADA accommodation requests → coordinator

DATA & SYSTEM INTEGRATIONS

- Translation services (NMT + memory)
- TTS / STT engines · multilingual
- TTY · screen-reader APIs
- CRM · language preference

BASELINE vs WITH AGENTIC SYSTEM

BASELINE	WITH AGENTIC
Non-English self-service <5%	Non-English self-service 60–75%
Interpreter scheduling Manual · slow	Interpreter on-demand Clinical edge only
Accessibility coverage Inconsistent	Accessibility coverage Default-on
Equity score Often unmeasured	Equity eval ≥ 0.9 Contractual

Referral handoff + care coordination

Close the loop — no patient lost between PCP and specialist.

WORKFLOW



USER ROLES

- PCP**
Originates referral
- Patient**
Schedules + attends
- Specialist**
Receives complete record
- Referral coord.**
Urgent + missing-data

HITL CHECKPOINTS

- Urgent referrals (red-flag) → coordinator priority queue
- Missing clinical data → PCP callback
- Closed-loop confirmation review

DATA & SYSTEM INTEGRATIONS

- Epic referrals workflow
- CCDA / HL7 — clinical document send
- Fax-to-FHIR ingestion
- Secure messaging · CRM

BASELINE vs WITH AGENTIC SYSTEM

BASELINE	WITH AGENTIC
<i>Referral leakage</i> 25–50%	<i>Referral leakage</i> <15%
<i>Time-to-appointment</i> 14–21 days	<i>Time-to-appointment</i> <7 days
<i>PCP loop closure</i> 30–55%	<i>PCP loop closure</i> 90%+
<i>Pre-visit prep done</i> Rare	<i>Pre-visit prep</i> Default

What changes when the lane lands

Headline deltas across the eight slices. Each row is one slice, with the most representative baseline-vs-agentic metric. These are not aspirational — they are the underwriting bar we propose to write into the eval contract.

#	SLICE	KEY METRIC	BASELINE		WITH AGENTIC
1	Slot search + intent	Online self-service	35–45%	→	70–85%
2	Reschedule + waitlist	Waitlist fill rate	8–12%	→	35–55%
3	Symptom triage + routing	Mis-routing rate	8–12%	→	<3%
4	Insurance + eligibility	Denial / rework rate	18–25%	→	<8%
5	Provider matching · continuity	PCP continuity	55–65%	→	80–90%
6	Reminders + no-show	No-show rate	15–30%	→	8–15%
7	Multi-language + accessibility	Non-English self-service	<5%	→	60–75%
8	Referral handoff + closure	PCP loop closure	30–55%	→	90%+

THE THROUGH-LINE

Every slice has a measurable, contractually-bindable delta. We don't need every slice to land — landing four out of eight is a transformed front door. The MVE proves Slice 1; the eval suite is the same shape across the others; the Healthcare Pod Harness compounds across deals.